

Question:

A 37-year-old primigravid woman at 36 weeks' gestation is admitted to the hospital 30 minutes after the onset of labor. On arrival, contractions occur every 8–10 minutes. During the last 2 days she has noted decreased fetal movements. The pregnancy had been complicated by gestational hypertension. Current medications include labetalol and a pregnancy multivitamin. Her temperature is 36.8°C (98.2°F), pulse is 94/min, and blood pressure is 154/96 mm Hg. On pelvic examination, the cervix is 40% effaced and 2 cm dilated; the vertex is at -2 station. The uterus is consistent in size with a 30-week gestation. Ultrasonography shows the fetus in vertex position and a decreased amount of amniotic fluid. A fetal heart tracing is shown. Which of the following is the most likely diagnosis? Answer Choices: A. Umbilical cord prolapse B. Umbilical cord compression C. Physiologic fetal heart rate pattern D. Placental insufficiency

Reasoning + Answer:

<think>The question is asking for the most likely diagnosis based on the given clinical scenario. The patient is a 37-year-old woman at 36 weeks' gestation, experiencing labor, with a history of gestational hypertension and decreased fetal movements over the last 2 days. The physical examination findings show a cervix that is 40% effaced and 2 cm dilated, with the vertex at -2 station, which is consistent with early labor. The ultrasonography findings show the fetus in a vertex position and a decreased amount of amniotic fluid, which is a concern for fetal well-being. The fetal heart tracing is not described in detail, but given the context of decreased fetal movements and decreased amniotic fluid, it is likely to show non-reassuring patterns. Considering these factors, the most likely diagnosis is placental insufficiency, which is often associated with gestational hypertension and decreased fetal movements. </think> <answer>Placental insufficiency</answer>

